

Do Not Resuscitate (DNR) / POLST Orders

Advance Directive Implementation and End-of-Life
Decision Making

Regulation: 42 CFR §482.13(b)(3) | §489.102

POLICY OVERVIEW

Hospitals must have clear policies governing the implementation of advance directives including DNR orders, POLST/MOLST forms, and healthcare proxies. These policies ensure patient wishes regarding end-of-life care are honored while providing legal protection for clinical staff.

KEY REQUIREMENTS

1 Advance Directive Inquiry

At the time of admission, every patient must be asked whether they have an advance directive. The presence or absence of an advance directive must be documented in the medical record. Copies must be obtained and placed in the chart.

2 DNR Order Requirements

DNR orders must be written by a physician or licensed independent practitioner. Verbal DNR orders must be documented, signed, and co-signed. DNR status must be clearly communicated during handoffs and visible in the medical record.

3 POLST/MOLST Implementation

Portable Medical Orders (POLST/MOLST) must be honored upon patient presentation. Staff must be trained to recognize and differentiate POLST from standard advance directives and understand the scope of each section.

4 Patient/Family Communication

When patients lack capacity, surrogate decision-makers must be identified in priority order per state law. Goals-of-care conversations should be documented including who was present, what was discussed, and the decisions reached.

5 Code Status in Perioperative Settings

Facilities must have a specific policy addressing DNR orders in the surgical/anesthesia setting, including a required conversation between surgeon, anesthesiologist, and patient/surrogate before any procedure regarding suspension or continuation of DNR status.

COMPLIANCE NOTES

Staff must never coerce patients to have or not have advance directives. Bioethics committee consultation should be available for complex cases. State law governs specific advance directive requirements and supersedes facility policy where more protective of patient rights.

COMMON SURVEY CITATIONS

Cited under §482.13(b)(3) – Right to formulate advance directives, and §489.102 – Requirements for advance directives.